

Diarrhea in Children

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Diarrhea is frequent, loose, or watery bowel movements (BMs) that differ from a child's normal pattern. It is a very common problem in children.

Identifying mild diarrhea may be difficult because in healthy children, the number and consistency of BMs vary with age and diet. For example, breastfed infants who are not yet receiving solid food often have frequent, loose stools that are considered normal. A sudden increase in number and looseness may indicate diarrhea in these infants. However, having watery stools for more than 24 hours is never normal.

Sometimes diarrhea contains blood or mucus. Children with diarrhea may lose their appetite, [vomit](#), lose weight, or have a [fever](#). If diarrhea is severe or lasts a long time, children are likely to lose a significant amount of body fluid ([dehydration](#)). Infants and young children can become dehydrated more quickly, sometimes in less than 1 day. Severe dehydration can cause seizures, brain damage, and death.

Worldwide, diarrhea causes 1.2 million deaths a year. In the United States, diarrhea accounts for approximately 9% of hospitalizations for children under 5 years old.

(See also [Diarrhea in adults](#).)

Causes of Diarrhea in Children

Likely causes of diarrhea depend on whether it lasts less than 2 weeks (acute) or more than 2 weeks (chronic). Most cases of diarrhea are acute.

Common causes

Acute diarrhea is usually caused by

- Infectious [gastroenteritis](#)

- [Food poisoning](#)
- Use of antibiotics
- [Food allergies](#)

Gastroenteritis is usually caused by a virus, but it can be caused by bacteria or a [parasite](#).

Food poisoning usually refers to diarrhea, vomiting, or both caused by eating food contaminated by toxins produced by certain bacteria, such as [staphylococci](#) or [clostridia](#).

Certain antibiotics can alter the types and number of bacteria in the intestine. As a result, diarrhea can occur. Sometimes using antibiotics enables a particularly dangerous bacteria, *Clostridioides difficile* (*C. diff*), to multiply. *Clostridioides difficile* releases toxins that can cause inflammation of the lining of the large intestine ([colitis](#)).

Chronic diarrhea is usually caused by

- Dietary factors, such as [lactose intolerance](#) or overconsumption of certain foods
- Infections (particularly those caused by [parasites](#))
- [Celiac disease](#)
- [Inflammatory bowel disease](#)

Less common causes

Acute diarrhea can also result from more serious disorders such as [appendicitis](#), [intussusception](#), and [hemolytic-uremic syndrome](#) (a complication of certain types of bacterial infection). These serious disorders are usually associated with other worrisome symptoms besides diarrhea, such as severe abdominal pain or swelling, bloody stools, fever, and ill appearance.

Chronic diarrhea can also result from disorders that interfere with the absorption of food ([malabsorption disorders](#)), such as [cystic fibrosis](#), and a [weakened immune system](#) (due to a disorder such as [HIV infection](#) or use of certain medications).

Diarrhea sometimes results from [constipation](#). When hardened stool accumulates in the rectum, soft stool may leak around it and into the child's underwear.

Evaluation of Diarrhea in Children

Warning signs

Certain symptoms are cause for concern. They include

- Signs of dehydration, such as decreased urination, lethargy or listlessness, crying without tears, extreme thirst, and a dry mouth, and weight loss
- Ill appearance
- High fever
- Blood in stool
- Pain in the abdomen and, when touched, extreme tenderness
- Bleeding in the skin (seen as tiny reddish purple dots [petechiae] or splotches [purpura])

When to see a doctor

Children with any warning signs should be evaluated by a doctor right away, as should those who have diarrhea and are not drinking or are drinking very little.

If children have no warning signs and are drinking and urinating normally, the doctor should be called if diarrhea lasts 2 days or more or if there are more than 6 to 8 episodes of diarrhea a day. If diarrhea is mild, a doctor's visit is generally unnecessary. Children with prolonged diarrhea should be seen by a doctor.

What the doctor does

Doctors first ask questions about symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause and the tests that may need to be done (see table

[Some Causes and Features of Diarrhea in Children](#)

Doctors ask what the BMs look like, how frequent they are, how long they last, and whether the child has other symptoms, such as fever, vomiting, or abdominal pain.

Doctors also ask about potential causes, such as diet, use of antibiotics, consumption of possibly contaminated food, recent contact with animals, and recent travel.

A physical examination is done, looking for symptoms of dehydration and disorders that can cause diarrhea. The abdomen is checked for swelling and tenderness.

Doctors also evaluate the child's [growth](#).



Some Causes and Features of Diarrhea in Children

Cause	Common Features*	Tests
Acute diarrhea (lasting less than 2 weeks)		
Antibiotic use	Recent use of antibiotics Often no other symptoms	A doctor's examination Sometimes tests for <i>Clostridioides difficile</i> toxin in stool
	Often with vomiting Dehydration common especially among infants and young children Sometimes fever and abdominal pain Rarely blood in stool	
<u>Gastroenteritis</u> due to viruses, bacteria, or parasites†	Sometimes recent contact with infected people (such as those at a day care center, at a camp, or on a cruise), with animals at a petting zoo (where <i>Escherichia [E.] coli</i> may be acquired), or with reptiles (which may be infected with <i>Salmonella</i> bacteria) or recent consumption of undercooked, contaminated food or contaminated water	A doctor's examination Sometimes examination and testing of stool
<u>Food allergy</u> or <u>food poisoning</u>	Food allergy: Hives, rash, swelling of the lips, vomiting,	A doctor's examination



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